

RED REPORT

For **splenic marginal zone lymphoma (SMZL)** treated with **rituximab monotherapy**, the best available data suggest:

- **Rituximab maintenance can prolong remission / improve PFS**, but
- **There's no proven overall survival (OS) benefit**, and
- **Ongoing rituximab exposure increases immune suppression risks** (notably hypogammaglobulinemia/infections), which matters a lot at age 82.

What the evidence says (most relevant to your scenario)

1) SMZL rituximab monotherapy cohorts (not randomized):

- In a large prospective/retrospective experience from the Kalpadakis group, many responders received **maintenance rituximab every ~2 months for 1–2 years**, and **PFS at 5 years was substantially higher with maintenance vs no maintenance (84% vs 36%)** (nonrandomized comparison, so selection bias is possible). ([PubMed](#))

2) Randomized data exist, but mostly after chemoimmunotherapy (not rituximab-only induction):

- The **BSH 2023/2024 guideline** cites the **MAINTAIN** randomized phase III trial (SMZL/NMZL) showing a **PFS advantage** for rituximab maintenance after induction (e.g., BR or R-CVP), **but no OS difference at 6 years**. ([BSH](#))
This supports the idea that maintenance delays relapse, but doesn't clearly help people live longer.

3) Guideline framing: maintenance is optional, not mandatory

- The accessible **NCCN patient guideline** explicitly notes that if treated with **rituximab alone**, you *might* have maintenance rituximab (i.e., it's presented as an option, not a requirement). ([NCCN](#))
- ESMO guidance similarly describes rituximab responses as long-lasting and states maintenance **may** improve PFS (again: “may,” not “must”). ([ScienceDirect](#))

4) Toxicity/immune risk with prolonged rituximab exposure

- In lymphoma populations, **maintenance rituximab is associated with higher rates of hypogammaglobulinemia**, and infection risk is a key tradeoff when there's no OS advantage. ([PMC](#))

Practical recommendation for *this* 82-year-old with PET CR after 4 weekly doses

It is very reasonable to **STOP rituximab now and observe**, with close follow-up, **given**:

- She achieved a **complete metabolic response** and is clinically improved.
- The main expected benefit of maintenance is **longer PFS / delayed relapse**, not proven OS benefit. ([BSH](#))
- At **82**, the “cost” side of maintenance (infections, hypogammaglobulinemia, infusion reactions, logistical burden) becomes more consequential. ([PMC](#))

When I’d lean *toward* maintenance anyway

Consider maintenance **only if** the care team believes she has a particularly high relapse risk *and* she’s likely to tolerate ongoing B-cell depletion (e.g., recurrent cytopenias before treatment that would be dangerous if they recur quickly, or very limited “reserve” for relapse).

If chosen, typical regimens in SMZL reports/guidelines are **time-limited** (often **q2 months for 1–2 years**)—not indefinite—paired with monitoring. ([PMC](#))

If you stop now: what “good” surveillance looks like

- Clinic + CBC/CMP every ~3 months initially (then space out if stable)
- Periodic imaging **only if symptoms/labs suggest relapse** (many clinicians avoid routine PETs in indolent lymphomas unless there’s a specific reason)
- If relapse occurs, **rituximab re-treatment is often effective** in SMZL, and other options remain available (including splenectomy or chemoimmunotherapy depending on fitness and disease behavior). ([PMC](#))

If you do maintenance: safety guardrails

- Baseline + periodic **IgG** monitoring (and clinical infection history), with low threshold to pause/stop if recurrent infections or significant hypogammaglobulinemia emerge. ([PMC](#))
- Keep vaccines up to date **before** additional rituximab doses when possible (response can be blunted during B-cell depletion)

Bottom line

For an **82-year-old in complete metabolic response after rituximab monotherapy**, **stopping therapy and observing is appropriate and commonly favored**, because maintenance primarily

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improves **time to relapse** without clear OS benefit, while increasing immune-related risks. Maintenance can be offered as an **optional, time-limited** strategy if delaying relapse is a top priority and infection risk is acceptable. ([PubMed](#))